Real-World Outcomes of Neurolix versus Triptan-Only Care in Episodic Migraine

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Treatments	Neurolix 60 mg daily prevention vs acute triptan therapy only
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Design

Retrospective fictional cohort study using electronic health records from 14 headache clinics. 1,860 adults with 4 to 14 monthly migraine days were included: 742 started Neurolix and 1,118 continued triptan-only care.

Outcomes

At 6 months, monthly migraine days decreased by 4.8 with Neurolix versus 3.1 with triptan-only care. A 50% or greater responder rate was achieved by 47% versus 29% of patients. Acute medication use decreased by 3.9 versus 1.2 days per month.

Persistence and tolerability

Treatment persistence at 6 months was 72%. The most common reason for stopping Neurolix was nausea (5.1% of patients), consistent with the pooled adverse event review.

Conclusion

Neurolix was associated with better migraine outcomes than triptan-only care in routine practice.

Summary table

6-month outcome	Neurolix (n=742)	Triptan only (n=1,118)
Reduction in monthly migraine days	4.8	3.1
>=50% responder rate	47%	29%
Reduction in acute medication days	3.9	1.2